

Eating Disorders Symptoms and Questions - Male (Adult)

Algorithm 1489 | Adult (18 years and older) | Gender at birth: Male

Definition

- Safety-first assessment of possible eating-disorder symptoms in adolescents and adults, including acute medical instability, suicide or self-harm risk, pregnancy, safeguarding, and need for confidential in-person assessment. This protocol does not diagnose an eating disorder.
- Adult patient.
- Male.

Initial assessment

1. [OPEN_TEXT] When safe and appropriate, speak privately with the patient. What restriction, binge eating, vomiting, laxative or diuretic use, water loading, excessive exercise, weight change, or reduced intake is occurring, and for how long?
2. [YES_NO] Is there fainting, inability to stand, chest pain, severe or irregular palpitations, breathing difficulty, confusion, seizure, severe weakness, very little urine, inability to keep fluids down, blood in vomit, or severe abdominal pain?
3. [YES_NO] Is there current suicidal intent, a suicide plan, recent self-harm, inability to stay safe, or an immediate threat from another person?
4. [OPEN_TEXT] Record age, pregnancy or possible pregnancy, diabetes or other illness, medicines, recent purging, fluid intake and urine output, and any known abnormal pulse, blood pressure, glucose, or electrolytes.
5. [YES_NO] Does the patient feel safe with the caregiver or person present, and can they speak freely? Consider coercion, forced eating or restriction, abuse, neglect, and safeguarding without confronting a suspected unsafe person.

Triage and redirect questions

1. Redirect → Suicide Concerns

Are thoughts of suicide or immediate self-harm the primary concern?

Suicidal thoughts or immediate self-harm risk belong in the Suicide Concerns pathway.

1. Disposition level 100

Is there collapse or fainting, inability to stand, chest pain, severe or irregular palpitations, breathing difficulty, confusion, seizure, severe weakness, severe dehydration or minimal urine, inability to keep fluids down, blood in vomit, severe abdominal pain, known severe electrolyte or glucose abnormality, current suicidal intent or plan, recent serious self-harm, inability to stay safe, or immediate danger from another person?

Eating disorders can cause life-threatening dehydration, electrolyte disturbance, hypoglycaemia, cardiac instability, organ failure, and psychiatric crisis. NICE recommends acute medical care for severe dehydration, electrolyte imbalance, malnutrition, or incipient organ failure; immediate suicide, self-harm, or safeguarding danger also requires emergency action.

1. Disposition level 70

Without an emergency feature, is there suspected restriction, binge eating, self-induced vomiting, laxative or diuretic misuse, water loading, excessive exercise, rapid or concerning weight change, growth or puberty concern, pregnancy, or coercion or safeguarding concern?

A suspected eating disorder requires prompt in-person medical and mental-health assessment; BMI, weight, or duration alone must not determine access to treatment. Adolescents need developmentally appropriate privacy and safeguarding, and pregnancy requires coordinated obstetric and eating-disorder care. The exact Qatar service and timeframe are governance-required.

Advice

Emergency eating-disorder or safety crisis

- Call Qatar 999 now for collapse, serious physical symptoms, or immediate suicide, self-harm, or violence risk. Do not allow self-driving. Keep the patient with a safe, trusted person, remove immediate means of harm only if safe, and follow the 999 call-handler's instructions. Do not attempt rapid feeding, forced feeding, electrolyte replacement, or large fluid intake outside a clinician-supervised plan because refeeding and electrolyte shifts can be dangerous. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: collapse, confusion, seizure, chest pain, severe palpitations, breathing difficulty, or reduced urine; suicidal action, escalating self-harm, or immediate danger.

Confidential in-person eating-disorder assessment

- Use the Qatar governance-approved in-person medical and mental-health pathway; the exact non-emergency service and timeframe are not defined by this UAT protocol. Arrange physical observations, hydration and electrolyte assessment, glucose when indicated, ECG risk assessment, mental-health and suicide screening, pregnancy care where relevant, and safeguarding review. Offer the adolescent or adult a private conversation while observing local consent and safeguarding policy. Do not prescribe a home refeeding, purging-withdrawal, fluid, electrolyte, or weight-restoration plan by telephone.
- Call Back If: fainting, chest pain, palpitations, confusion, severe weakness, vomiting blood, inability to drink, or reduced urine; suicidal thoughts, self-harm, coercion, abuse, or inability to remain safe.

First aid

- Call Qatar 999 now for collapse, serious physical symptoms, or immediate suicide, self-harm, or violence risk. Do not allow self-driving. Keep the patient with a safe, trusted person, remove immediate means of harm only if safe, and follow the 999 call-handler's instructions. Do not attempt rapid feeding, forced feeding, electrolyte replacement, or large fluid intake outside a clinician-supervised plan because refeeding and electrolyte shifts can be dangerous. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Eating disorders", <https://www.nhs.uk/conditions/eating-disorders/> (page last reviewed 23 January 2024)

- NICE NG69, "Eating disorders: recognition and treatment",
<https://www.nice.org.uk/guidance/ng69/chapter/recommendations> (accessed 25 July 2026)
- Qatar Ministry of Public Health, "Healthcare Services in Qatar",
<https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (999 ambulance access; accessed 25 July 2026)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: aa7edb4acd42d444a020089ca6743a05f40f132337176814d120a0d2f2f97892 | Status: BLOCKED_BY_RESEARCH_GAP